NeurAegis

THE FUTURE OF NEURONAL DEATH PREVENTION & NEUROLOGICAL DISORDERS TREATMENTS

Series A Fundraising - Investors Deck

ABOUT US

NeurAegis

A late preclinical stage neuropharmaceutical company founded in 2016

Pioneering the development of **first-in-class** compounds, specifically designed to treat major neurological disorders, focusing on **selective Calpain-2 inhibitors**

Our product portfolio addresses significant **unmet medical needs, targeting serious and growing neurological conditions**

THE NEURAEGIS OPPORTUNITY

POTENTIAL TO BE FIRST-IN-CLASS TO PREVENT NEURONAL DEATH

NeurAegis

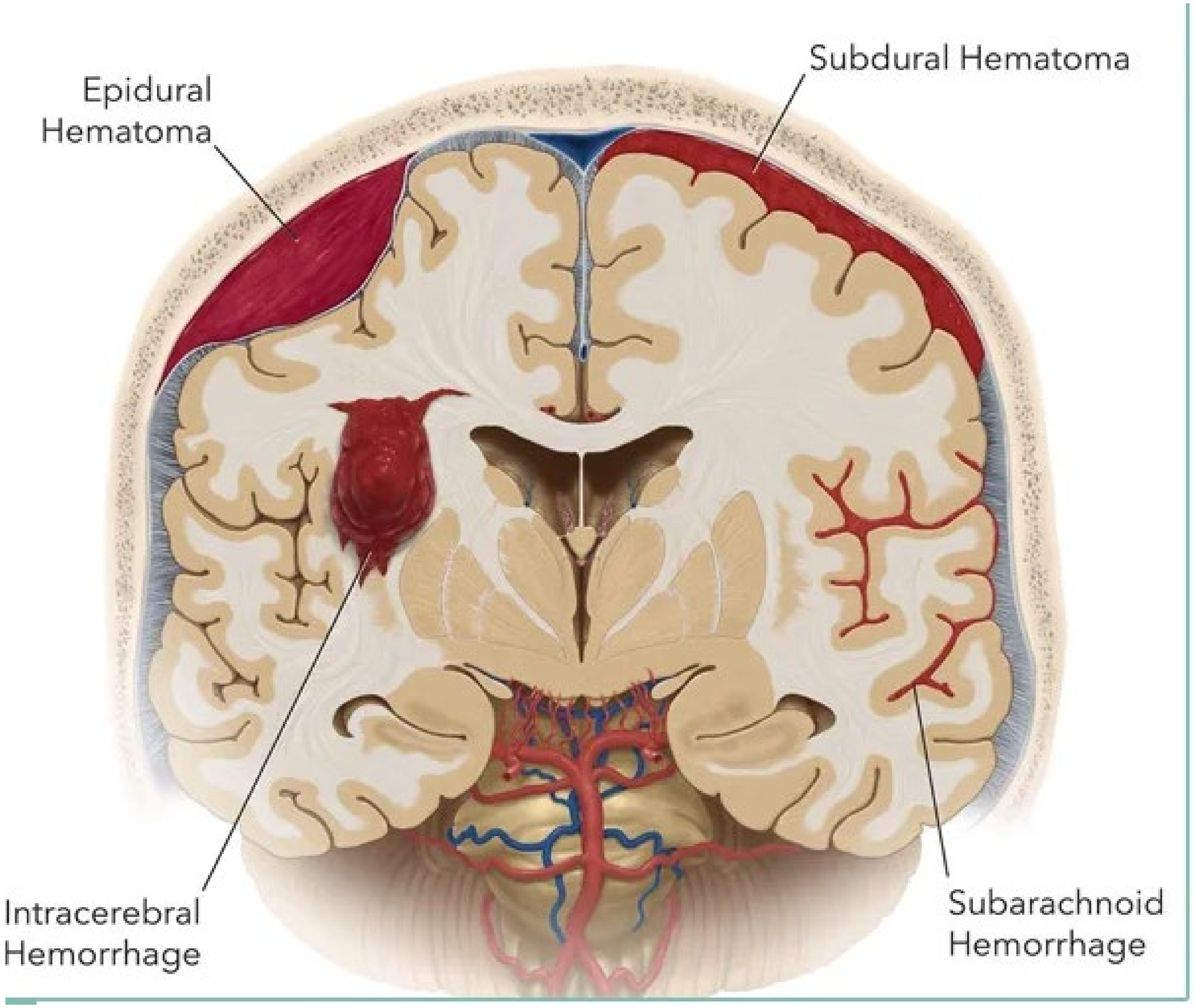
Portfolio of novel, first-in-class protease inhibitors & potentially transformative therapies for multiple neurological disorders with long-term disabling neurodegenerative brain effects

- **Evident unmet medical need**
- Blockbuster potential as first-in-class assets (Ozimpic-like drug)
- Large addressable markets **with broad opportunities & target indications**
  - Lead indications are concussion (where there is currently **no FDA approved medication**) and epilepsy
  - **Broad applicability** in other indications, such as Loud Noise-Induced Hearing Loss, Glaucoma, Alzheimer's Disease, and Parkinson's Disease

- Strong intellectual property portfolio
- Increased likelihood of clinical success:
  - Well-defined mechanism of action
  - **High protease selectivity**
  - Strong clinical hypothesis
  - **Non-invasive plasma biomarkers** reflecting brain target engagement
- Experienced, proven leadership team

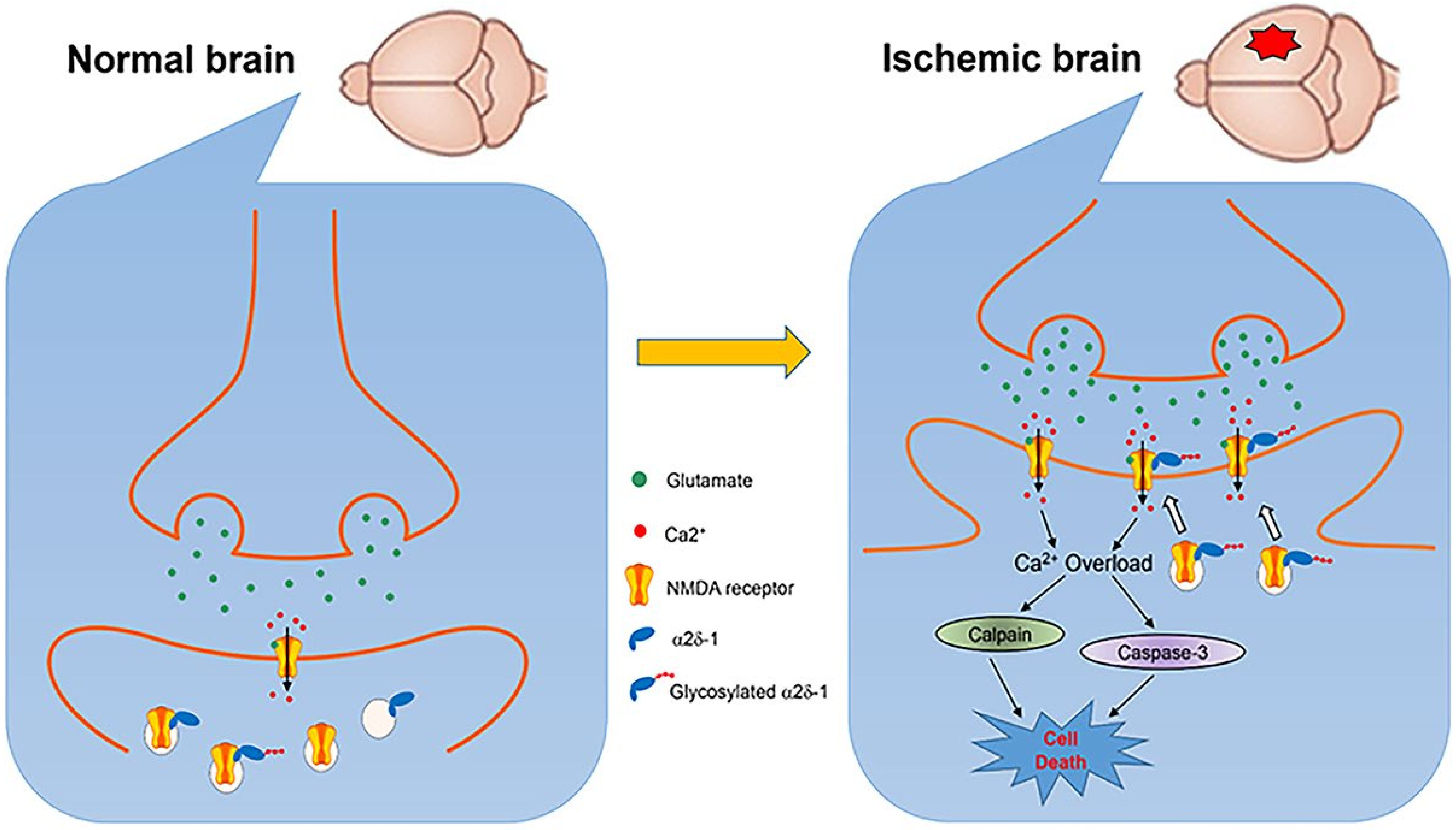
RAISING $10M SERIES A TO FUND TO KEY MILESTONE/ VALUE INFLECTION POINT OF COMPLETION OF PHASE 1 (IN CONCUSSION/TBI)

Subarachnoid hemorrhage (SAH)-induced vasospasm and stroke



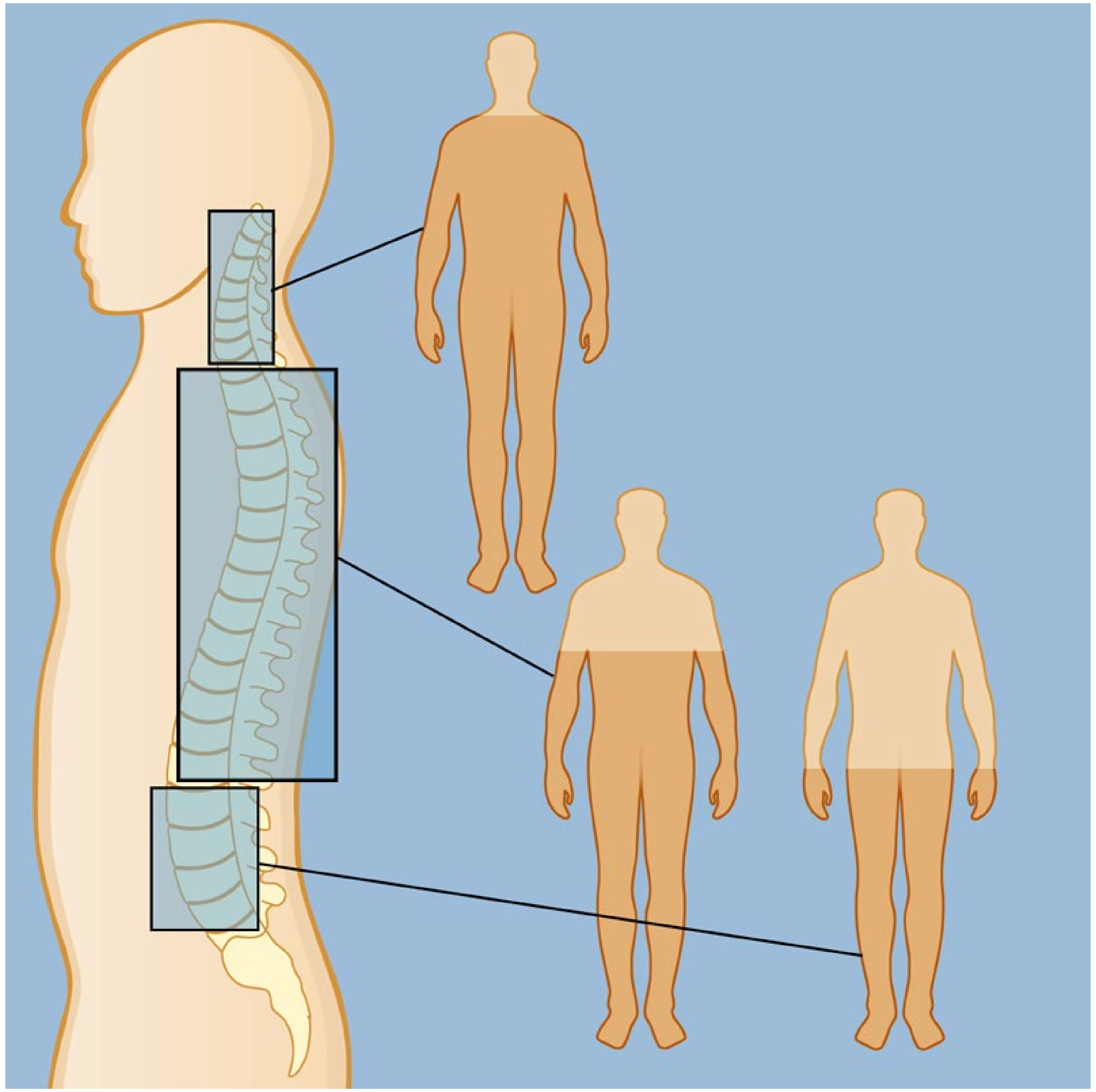
Subarachnoid hemorrhage (SAH) has an estimated incidence of 6 to 15 cases per 100,000 person-years. While it can occur at any age, it is more common in individuals between 45 and 70 years old, with a higher prevalence in women and Black individuals. SAH is a serious condition, with a high fatality rate and significant long-term disability for survivors.

Role of calpain-2 in SAH



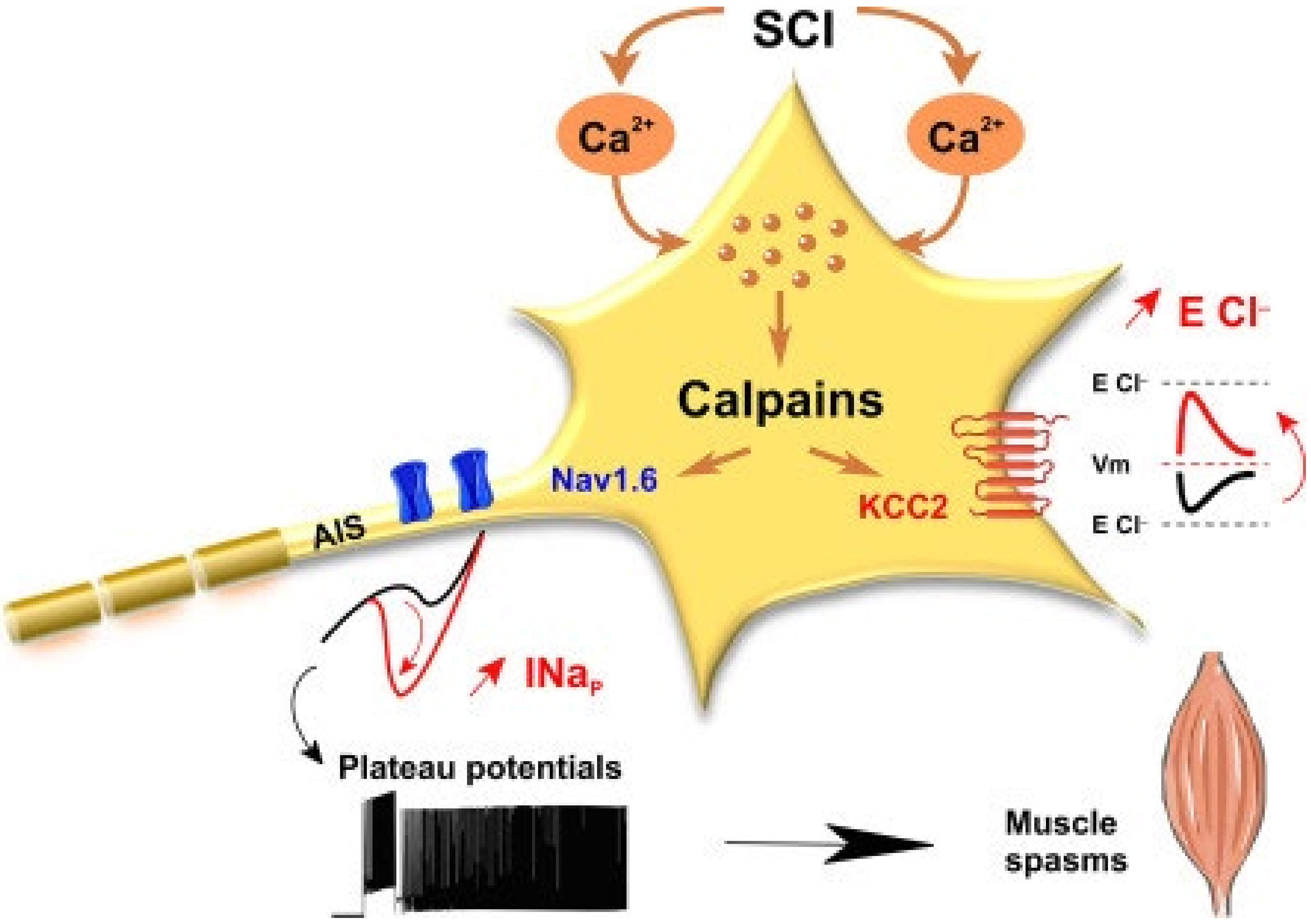
Calpain activation plays a significant role in brain damage following subarachnoid hemorrhage (SAH), particularly in the early stages of injury. Calpains are calcium-activated proteases that, when activated after SAH, contribute to neuronal cell death and blood-brain barrier (BBB) disruption. Calpain-2 inhibitors have shown promise in reducing brain injury and improving outcomes in animal models of SAH, suggesting a potential therapeutic target.

Spinal cord injury (SCI)



In the United States, approximately 17,000 new spinal cord injuries (SCI) occur each year, and there are an estimated 282,000 people living with SCI according to the National Institutes of Health (NIH). The annual incidence is around 54 cases per million people. While historically, SCI primarily affected younger men, the average age at injury has increased to 43, and there's a growing proportion of older adults experiencing SCI due to age-related factors.

Calpain and spinal cord injury



Calpain, a calcium-activated protease, plays a significant role in the secondary damage following spinal cord injury (SCI). Increased calpain activity, due to calcium influx after injury, leads to the breakdown of essential proteins, contributing to neuronal cell death and impaired function. Targeting calpain-2 with inhibitors is being explored as a potential therapeutic strategy for SCI.

OUR DISCOVERY

FOCUSING ON PREVENTING NEURONAL DAMAGE

NeurAegis

NEURODEGENERATIVE DISORDERS

Acute	Chronic
SAH-induced vasospasm/stroke	Epilepsy
SCI	Parkinson’s
Concussion	ALS
Ischemia	Huntington’s
Seizures	Alzheimer’s

The protease calpain-2 has been implicated in several neurodegenerative disorders

DISORDER

CALPAIN-2 ACTIVATION IN BRAIN NEURONS
(leads to cell death)

Calpain-2 activation plays a major role in neurodegeneration and leads to cell damage and death

TREATMENT

CALPAIN-2 DEACTIVATION
(Inhibiting Neurodegeneration)

Our selective calpain-2 inhibitors prevent neurodegeneration in multiple animal models of human diseases

NOVEL SCIENCE ON THE VERGE OF CLINICAL PROOF OF PRINCIPLE

INITIAL TARGET INDICATIONS

	SAH-induced vasospasm/stroke	SCI	Glaucoma
Current Therapies	Current treatment: nimodipine	Currently no FDA approved neuroprotective medications	Currently no FDA approved medications
Overall Incidence	**6 to 15 cases per 100,000 person-years**	**17,000 new spinal cord injuries (SCI) occur each year**	CDC estimates 0.6% of the global population
Development Status	Lead candidate selected & Initiating rat SAH model	Planning testing NA-184 in rat models of SCI	Discovery
Current Funding	100% funded NeurAegis	NINDS submission	Self Funded

CALPAIN-2 HAS BEEN IMPLICATED AND IS STRONGLY ACTIVATED IN EACH OF THESE DISORDERS

TARGETING NEUROLOGICAL DISORDERS

ADDRESSING HUGE UNMET MEDICAL NEED WITH NO FDA-APPROVED THERAPIES

NeurAegis

Clear Problem

- TBI is the leading cause of **death and disability** among children and young adults in the U.S.
- **> 69,000 TBI-related deaths** in the US in **2021**

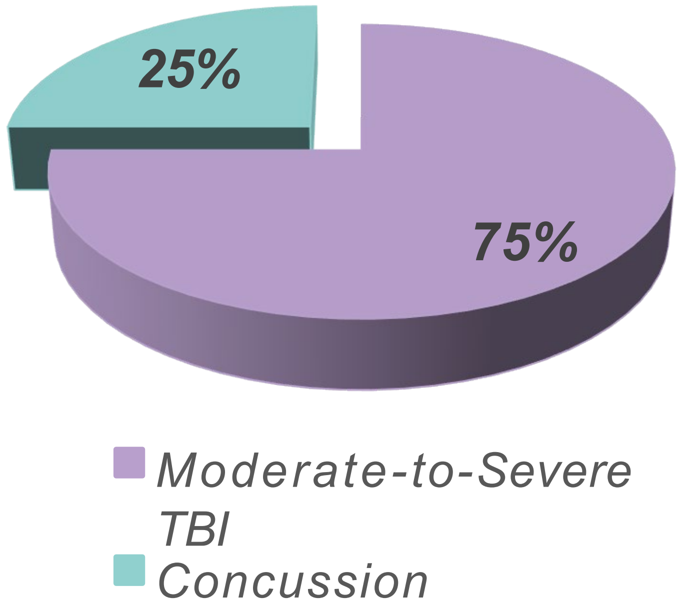
Large Market

- Over **3 million** Americans sustain a TBI each year; annual medical costs **> $48 billion**
- Over **3.8 million sports-** and **recreation-related concussions** per year in the US*
- **Epilepsy drug market** to reach over **$5 billion** by **2024****

Great Opportunity

- **No FDA-approved** therapies for TBI; estimated global market potential is **>$4 billion**
- Enormous markets for AD, PD and other indications that could be co-developed with big pharma partners

>3 Million Americans Sustain a TBI Annually



** Source: www.brainline.org)*
*** Source: Grand View Research*

TBI/CONCUSSION
SOLUTION

NeurAegis

PREVIOUS CLINICAL TRIAL FAILURES

CLEAR UNDERSTANDING OF All CHALLENGES

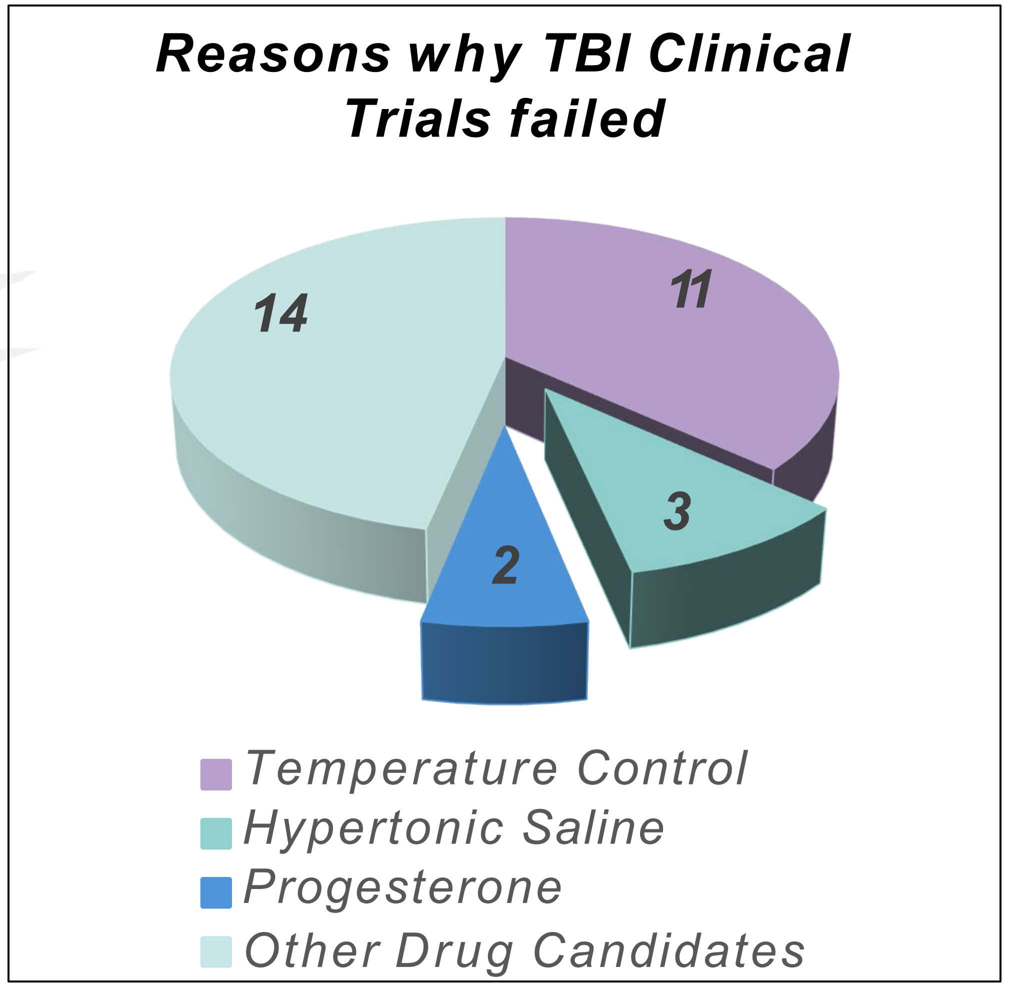
Causes of the failures of all traumatic brain injury trials since 1993:

» Unknown mechanism for neuronal damage

» Lack of quantitative and predictive biomarkers to guide trials

» Subjective outcome measures

» Patient recruitment challenges



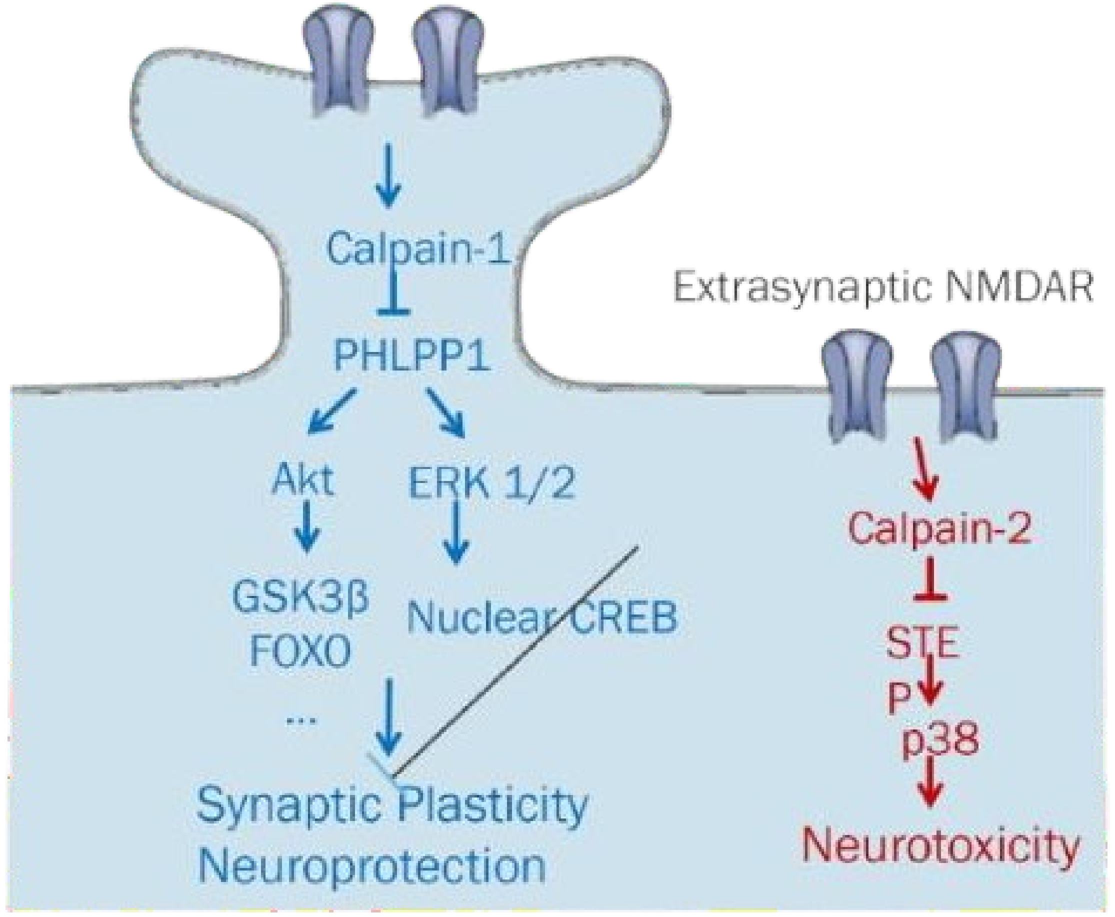
WE ARE ADDRESSING EACH OF THESE CHALLENGES IN OUR CLINICAL PLAN WITH THE GOAL OF OBTAINING THE FIRST FDA APPROVED THERAPY FOR CONCUSSION

TARGET

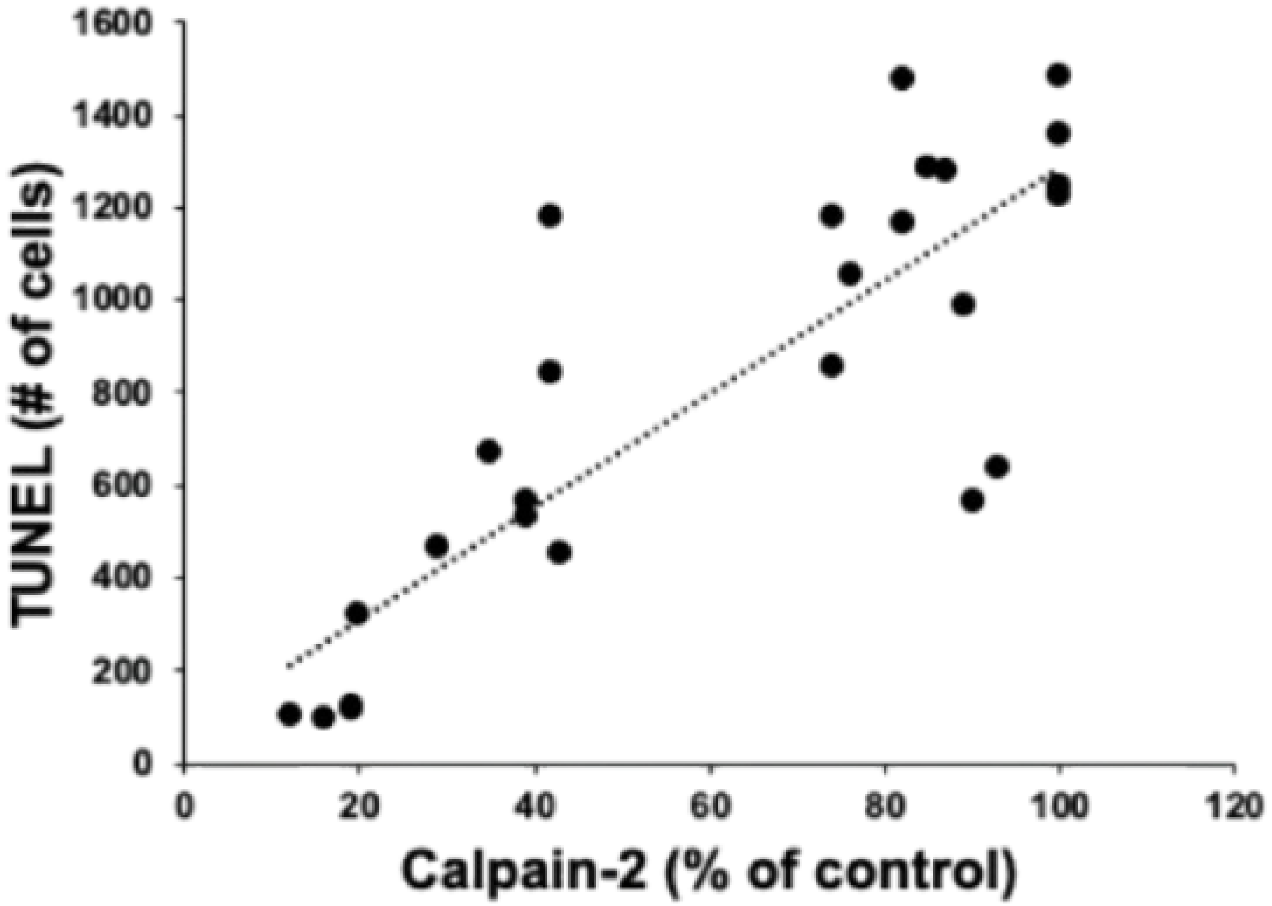
OUR PATH TO PREVENTING NEURONAL DAMAGE AFTER CONCUSSION

Our Key Discoveries	***Initial Validation of our Solution***
Identified a critical ***mechanism*** of neuronal damage and death resulting from concussion	We demonstrated that calpain-2 activation is ***correlated with cell damage and death*** following concussion

Calpain-1 and Calpain-2 play opposite functions in plasticity and neurodegeneration



Correlation of Cell Death and Calpain-2 Activity in Rat and Mouse Brain After TBI



INHIBITOR

OUR PATH TO PREVENTING NEURONAL DAMAGE AFTER CONCUSSION

NeurAegis

Our Key Discoveries	*Initial Validation of our Solution*
Discovered an ***inhibitor*** of this neurodegenerative process	Identified novel calpain-2 inhibitors that provide significant protection when administered after concussion (animal models)

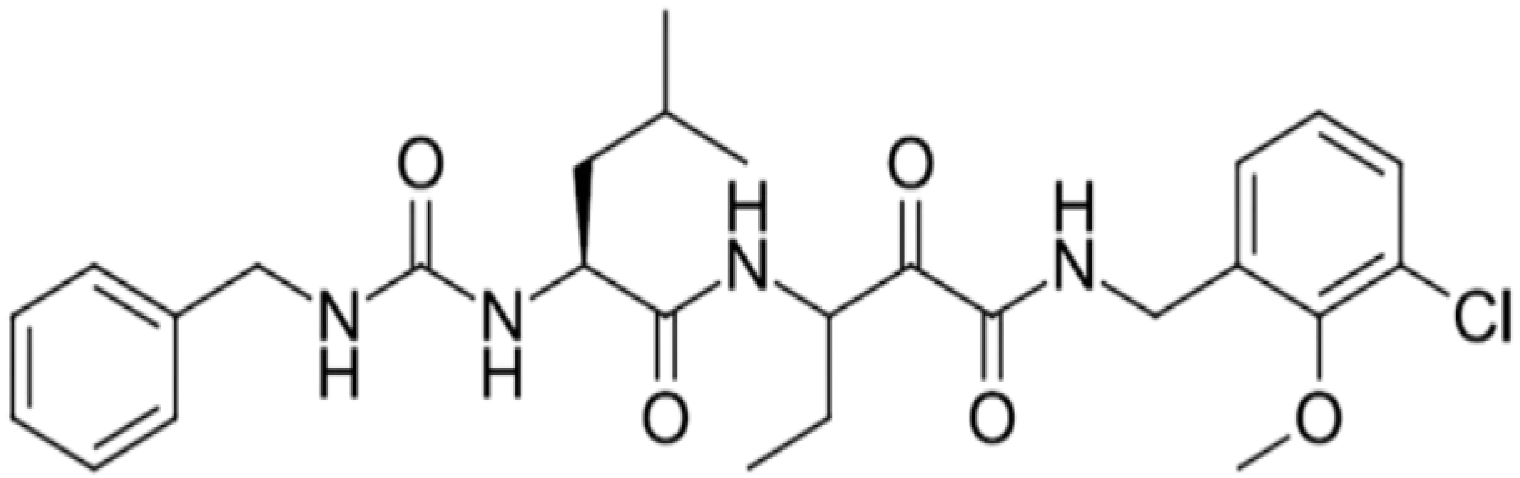
Calpain-2 inhibitor effects

Selective and potent calpain-2 inhibitor has **2 effects**:

» Enhances synaptic plasticity/ Learning and Memory

» Prevents neurodegeneration

NA-184 structure



Ref: Appendix p 35, 36

NA-184 is very potent and selective for Calpain-2 when assayed against Human Calpains *In Situ*

NA184 was tested on calpain activity in homogenates of HEK cells expressing either calpain-1or calpain-2, using spectrin degradation as the read-out

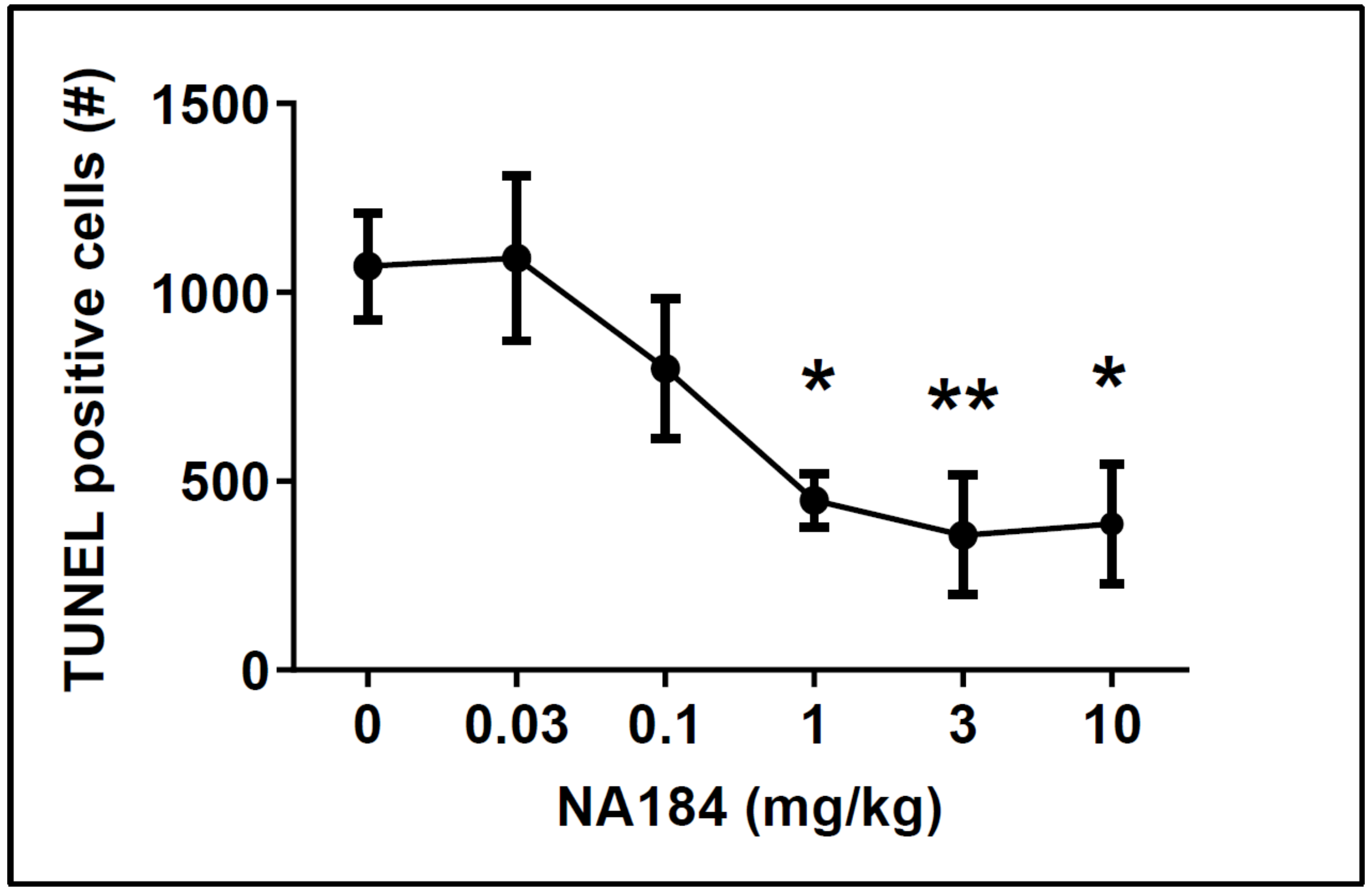
IC_{50} for human calpain-1: no inhibition up to 10µM
IC_{50} for human calpain-2: 1.2 nM

Ratio of selectivity: >3,000 fold

The cellular homogenate is closer to the in vivo situation as it contains the endogenous substrates of the two enzymes

Activity & Target engagement

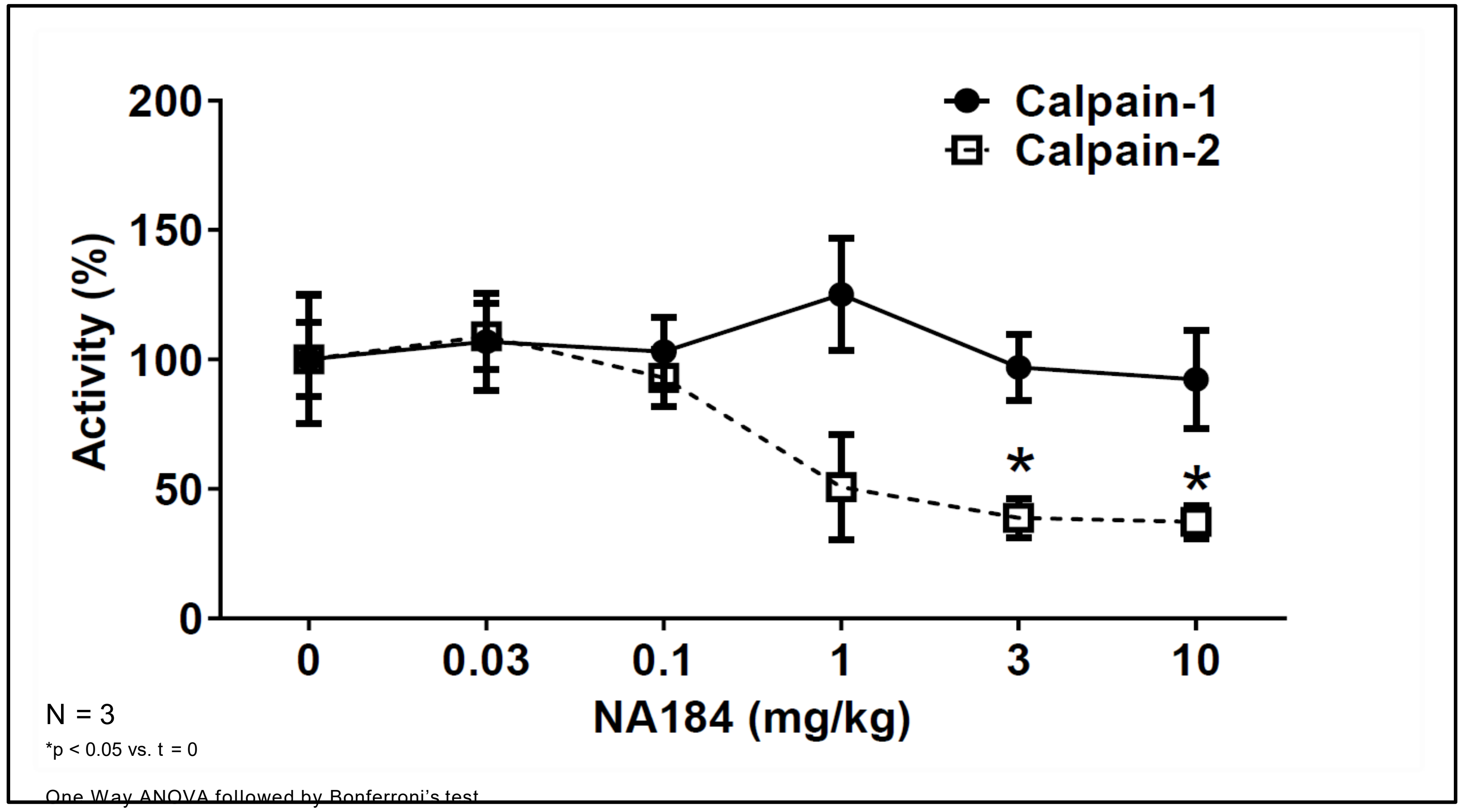
Intraperitoneal (IP) Injection of NA-184 1h After TBI Significantly Reduces Cell Death 24 h After TBI in Male Mice



N = 6 for 0.1 mg/kg; N = 3 for other groups. *p < 0.05 and *p<0.01. One Way ANOVA followed by Bonferroni's test

ED_{50} is approx. 0.13 mg/kg

IP injection of NA-184 1h After TBI Reduces Calpain-2 But Not Calpain-1 Activation in Brain 24 h After TBI in Male Mice



Cerebellar P2 fraction collected 24 h after TBI was used for measuring inhibitory activity of compounds against mouse endogenous calpain-1/-2

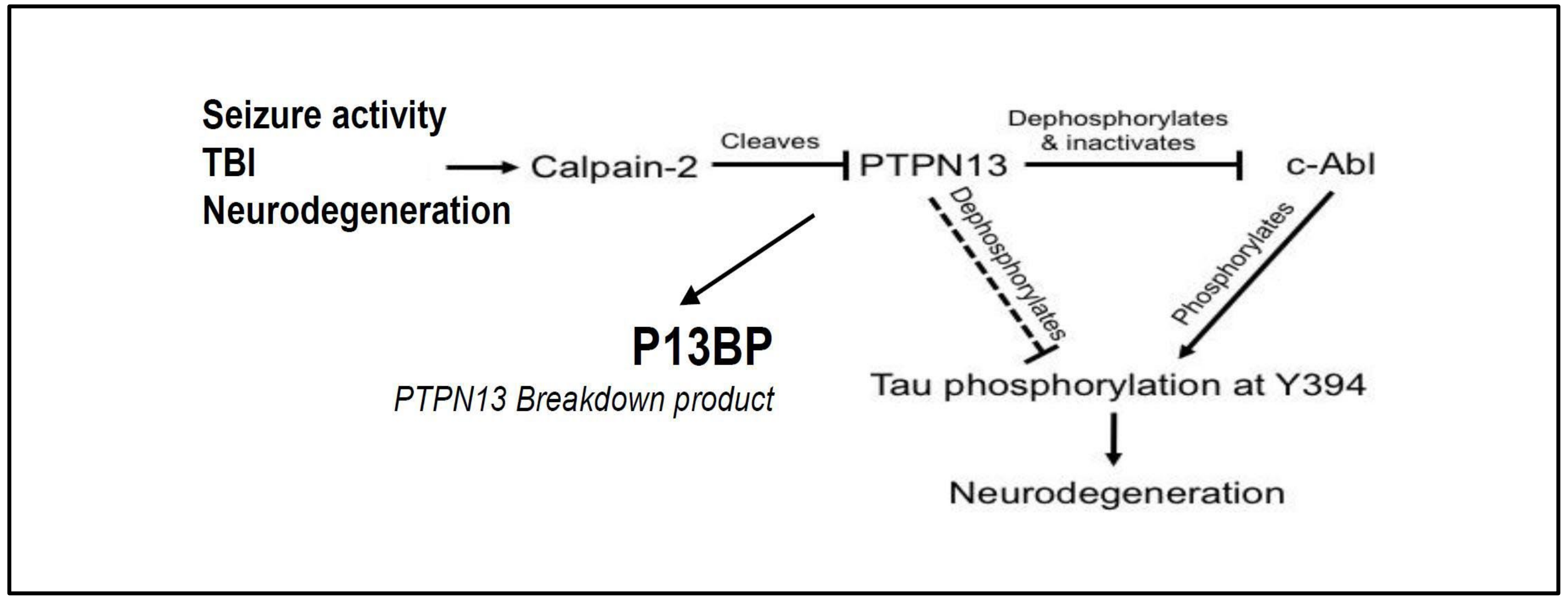
- Calpain-1 activity = calpain activity with 20 μM Ca^{2+} - calpain activity with no Ca^{2+}
- Calpain-2 activity = calpain activity with 2 mM Ca^{2+} - calpain activity with 20 μM Ca^{2+}

Ref: Appendix p 35, 36

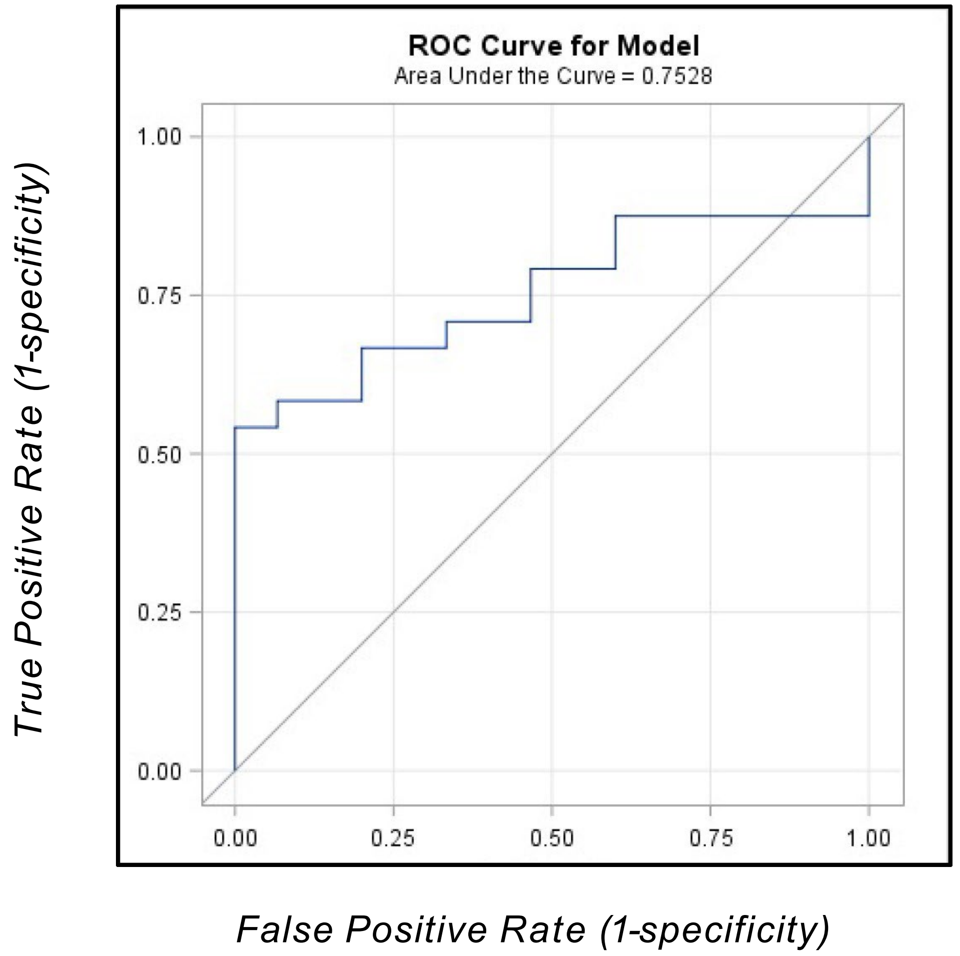
OUR PATH TO PREVENTING NEURONAL DAMAGE AFTER CONCUSSION

Our Key Discoveries	*Initial Validation of our Solution*
We identified **blood-based biomarkers** that directly correlates with cell death and drug target engagement	Discovered proprietary plasma biomarker (P13BP), which reflects calpain-2 activation in the brain (using human clinical samples in studies)

P13BP as a Blood Biomarker for TBI



Receiver Operative Characteristics (ROC) Curve For Human Plasma P13BP Levels



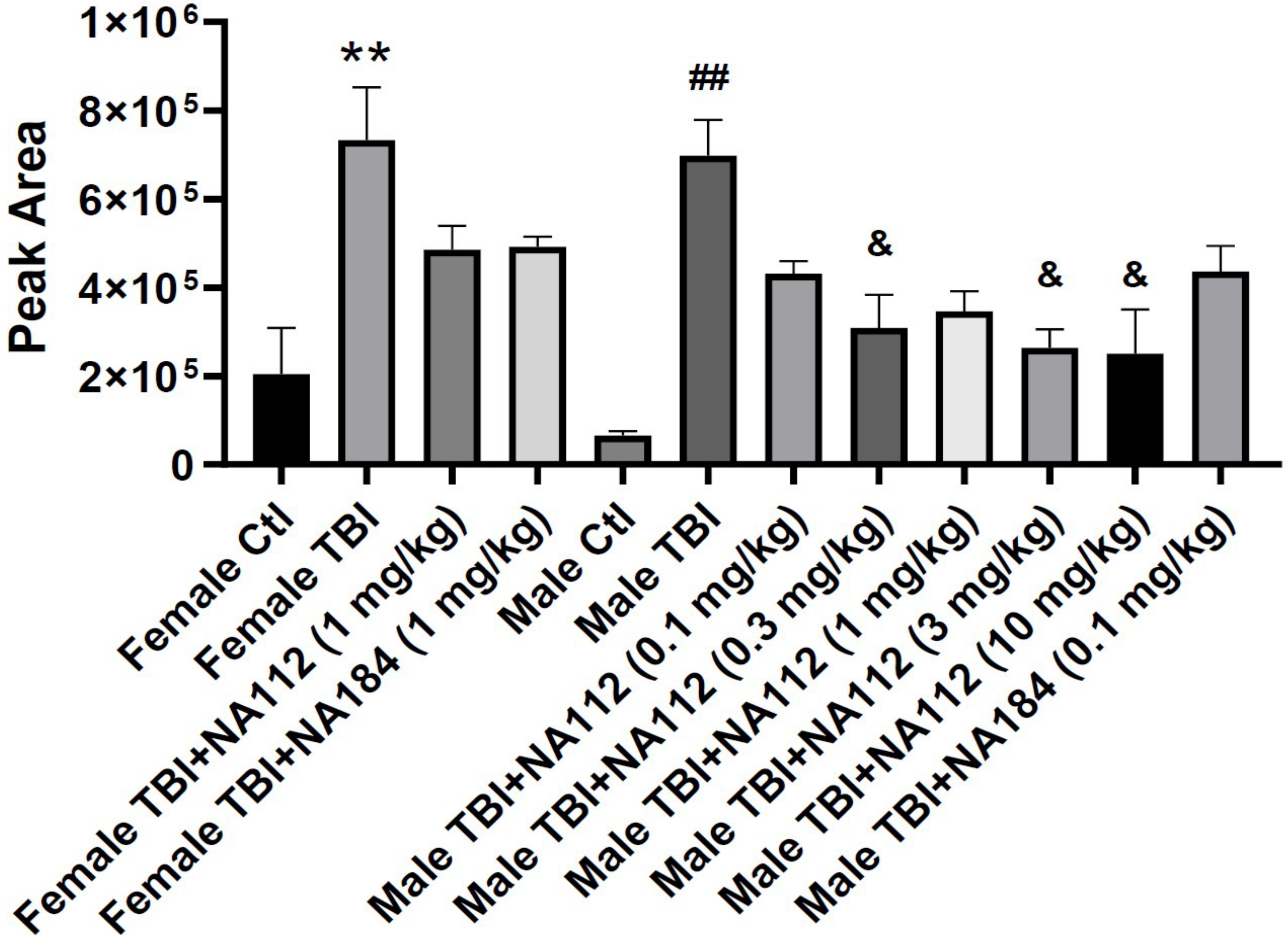
P13BP plasma level is a good predictor of TBI severity in humans

Ref: Appendix p 35, 36

NA-184

PREVENTS TBI-INDUCED INCREASE IN PLASMA P13BP

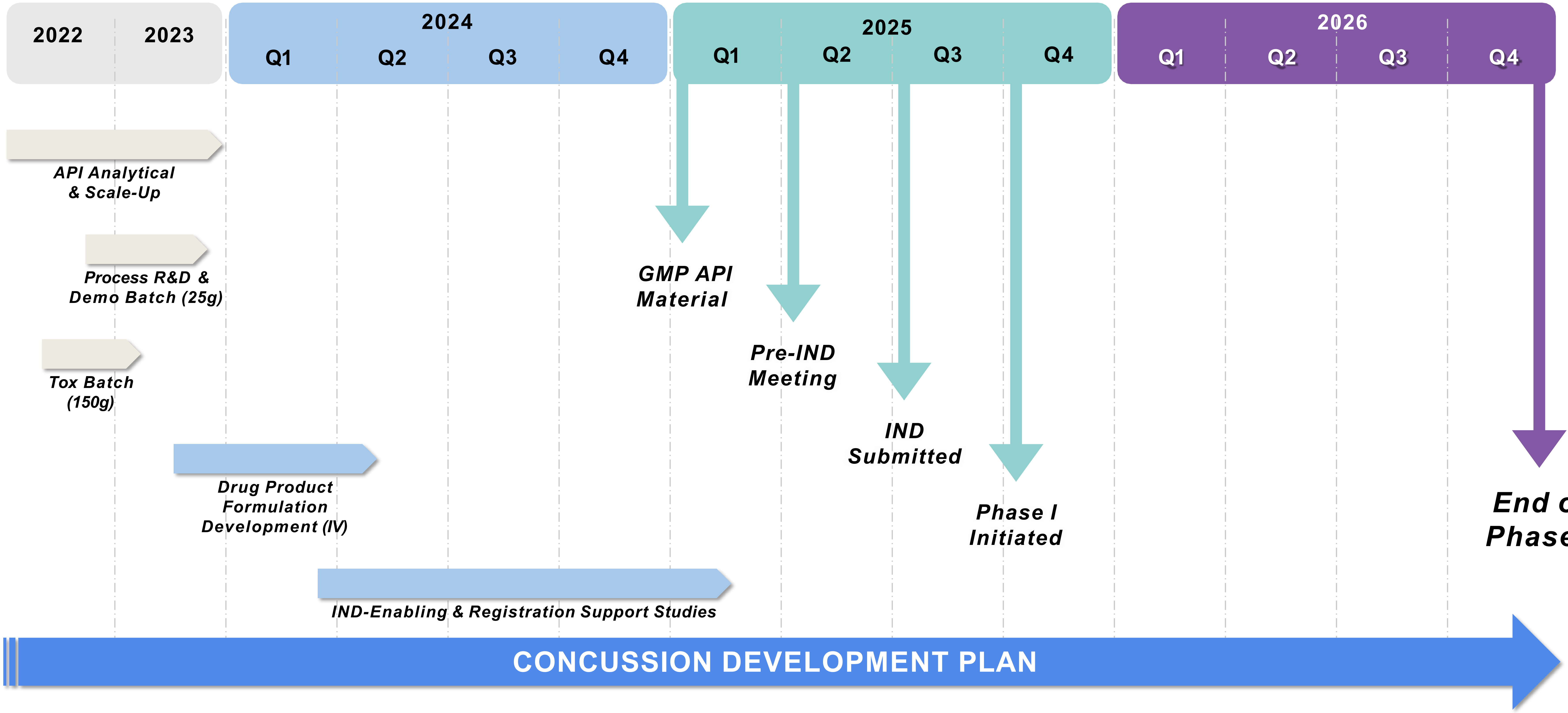
P13BP LEVELS ARE SIGNIFICANTLY INCREASED IN THE PLASMA OF MALE AND FEMALE MICE 24 H AFTER TBI; IP INJECTION OF NA-112 OR NA-184 REDUCES THE INCREASE IN P13BP AFTER TBI



NA-184 prevents TBI-induced increase in plasma P13BP

** : $p < 0.01$ vs Control females ##: p <).01 vs Control males &: $p < 0.05$ vs Control males

CONCUSSION DEVELOPMENT PROGRAM (2022-2026)



NEURAEGIS LEADERSHIP

NEURAEGIS LEADERSHIP TEAM

PROJECT LED BY RENOWNED EXPERTS IN NEUROSCIENCE

NeurAegis

Top Management

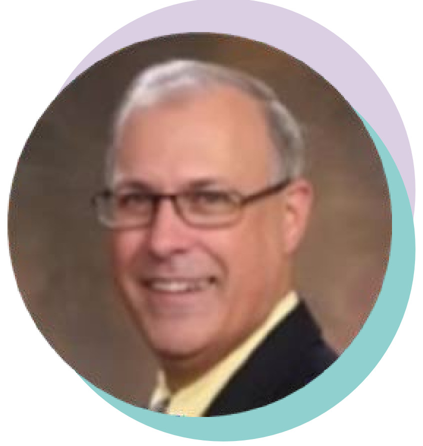
Michel Baudry, PhD
Founder & Chief Scientific Advisor
World-renowned neuroscientist with over 400 publications

Stella M. Sung, Ph.D.
President & CEO
C-Level operating executive & VC, helped build multiple life science firms

Michael G. Palfreyman, PhD, DSc
Drug Development Advisor
Expert in preclinical drug development

Key Managers

Philippe Bey, Ph.D.
Chemistry
ArQule, Sanofi Aventis, Hoechst Marion Roussel
Ph.D. from University Louis Pasteur (chemistry)



Greg Coulter, Ph.D.
Biological Chemistry
University of Guelph, Canada
President, CTM Solutions, LLC
Pharmaceutical Development Specialist

Kathy Fosnaugh, Ph.D.
Program Management
Aminex Therapeutics, Sirna Therapeutics, Ligand Pharmaceuticals
Ph.D. from Cornell University (microbiology)`

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Top Management

Michel Baudry, PhD

Founder & Chief Scientific Advisor

World-renowned neuroscientist with over 400 publications

- World-renowned neuroscientist, life science entrepreneur and professor. Co-Founder of 5 biotechnology companies across two countries.
- Identified calpain-2's role in neurodegeneration and discovered novel calpain-2 protease inhibitors. Earned his Ph.D. (Biochemistry) from University Paris VII (Paris, France)

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Stella M. Sung, Ph.D.

President & CEO

C-Level operating executive & VC, helped build multiple life science firms

- C-level (CEO/CBO) life Science/biotech executive with 25 years of experience in both operating and venture capital roles.
- Successful track record in negotiating, establishing, and executing transactions, agreements, and partnerships.
- Extensive experience in raising capital, building companies; served on or chaired Boards of >10 life science companies. Earned her Ph.D. (Chemistry) from Harvard University (National Science Foundation Pre-Doctoral Fellow).

in

Michael G. Palfreyman, PhD, DSc

Drug Development Advisor

Expert in preclinical drug development

- Scientific leader in pharmaceutical industry for over 40 years.
- Internationally recognized leader in neuropharmacology and drug development and clinical Proof of Concept.
- Successfully led multiple IND filings and clinical programs.
- Holds a D.Sc. and a Ph.D. degree in Pharmacology, both from the University of Nottingham, UK.

in

Key Managers

Philippe Bey, Ph.D.

Chemistry

ArQule, Sanofi Aventis, Hoechst Marion Roussel

Ph.D. from University Louis Pasteur (chemistry)

- President of Bey Consulting, Inc. Participated in and led teams responsible for over 30 INDs across several therapeutic areas, including oncology.
- Oversaw bulk drug supply for preclinical and clinical studies., Post-doctoral training at the California Institute of Technology.
- Adjunct professor of medicinal chemistry at the University Louis Pasteur for 12 years.
- Authored nearly 100 publications, 21 book chapters, and holds 63 issued patents.Inventor of difluoromethylornithine (DFMO), the standard of care for African sleeping sickness.

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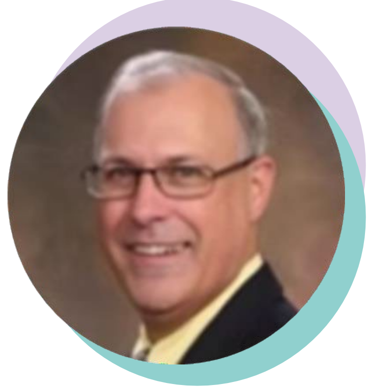

Greg Coulter, Ph.D.

Biological Chemistry

University of Guelph, Canada

President, CTM Solutions, LLC

Pharmaceutical Development Specialist

- A consultancy specializing in CMC and clinical trial materials.
- Over 20 years in GLP, GMP, and GCP regulated drug product development. Specializes in drug substance synthesis, scale-up, process development, analytical characterization, and regulatory CMC guidance.
- Postdoctoral training at the University of Denver. Previous Roles: Held positions at Synthetech, Cell Therapeutics, and SNBL USA, with increasing responsibilities. Recent Work: Contributed to the FDA approval of Contrave (weight loss drug) and Zydelig (PI3K delta inhibitor).

in

Kathy Fosnaugh, Ph.D.

Program Management

Aminex Therapeutics, Sirna Therapeutics, Ligand Pharmaceuticals

Ph.D. from Cornell University (microbiology)`

- Biotech consultant specializing in preclinical and Phase 1clinical program management and drug development at KL Fosnaugh Consulting, LLC
- Over 30 years of experience in translational research and development of small molecule and nucleic acid therapeutics.
- Has served as Program Manager for three IND submissions and Phase 1clinical trials.
- Holds a BS in microbiology from the University of Illinois and a PhD in microbiology from Cornell University.
- Completed postdoctoral research at the University of California, San Diego. Certified in Biomedical Regulatory Affairs from the University of Washington

in

SCIENTIFIC & CLINICAL ADVISORY BOARD

PROJECT SUPPORTED BY WORLWIDE RECOGNIZED PROFESSIONALS

NeurAegis

Cleveland Clinic

Dr. Imad Najm, MD

Director of the Epilepsy Center
Cleveland Clinic

- » Director of the Epilepsy and Clinical Neurophysiology Fellowship Program at Cleveland Clinic (2000-2003).
- » Head of the Section of Adult Epilepsy at Cleveland Clinic (2003).
- » Medical Doctor degree from Saint Joseph University, Beirut, Lebanon (1988).
- » Post-Doctoral Research Fellow in Neurobiology at the University of Southern California (1989-1992), focused on mechanisms of status epilepticus-induced neuronal injury.
- » Certified in adult neurology by the American Board of Psychiatry and Neurology (1998).Added qualification in clinical neurophysiology (1999).

in

RVCL Research
Penn Medicine

Dr. Ramon Diaz-Arrastia, MD, PhD

Director, Traumatic Brain Injury Clinical Research Center The University of Pennsylvania
Perelman School of Medicine

- » Attending Neurologist at the Hospital of the University of Pennsylvania and Penn Presbyterian Medical Center
- » His research focuses on the molecular mechanisms of neuronal injury, neurodegeneration, traumatic brain injury, and epilepsy.
- » Professor of Neurology at the Uniformed Services University of Health Sciences and Investigator at the National Institute of Neurological Disorders and Stroke. (2011-2016)
- » Adjunct Professor at the University of Texas at Dallas.(2004)
- » MD from Baylor College of Medicine (1988)
- » Authored numerous articles in peer-reviewed journals, primarily focused on neurodegeneration, traumatic brain injury, and post-traumatic epilepsy.

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RANCHO LOS AMIGOS

Barry Jordan, MD, MPH

Chief Medical Officer
Ranchos Los Amigos Nat Rehab Center

- » Assistant Medical Director and Interim Chief Medical Officer at Burke Rehabilitation Hospital, White Plains, NY.
- » Team physician for U.S.A. Boxing.
- » Chief Medical Officer for the New York State Athletic Commission (20 years).
- » B.A. in Neurophysiology from the University of Pennsylvania.
- » MD from Harvard Medical School.
- » MPH from Columbia University

in

BUSINESS ADVISORS

PROJECT SUPPORTED BY EXPERIENCED EXPERTS IN STRATEGY & MANAGEMENT

MINTZ

Lewis Geffen

Corporate Counsel; Co-Chair Life Science Practice, Mintz Levin

- A corporate and licensing attorney focused on the life sciences, with decades of experience helping companies in the sector tackle strategic issues and complete complicated transactions.
- He is the go-to outside lawyer for preeminent biotechnology, pharmaceutical, medical device, and medtech companies, shepherding them through financing and licensing deals that fund drug development.
- Active member of the Biotechnology Industry Organization, Massachusetts Biotechnology Council, BioCOM, AdvaMED, MassMEDIC and other professional organizations. He is also a member of the American and Boston Bar Associations

Fox Rothschild

Peter Corless

IP Counsel; Partner, Fox Rothschild LLP

- Strategic adviser on patent and trademark portfolio management and intellectual property due diligence for businesses, academic institutions, and investors in North America, Europe, and Asia.
- Works with academic and medical research institutions, biotech, pharmaceutical, manufacturing companies, venture capital firms, underwriters, and medical technology companies.Previous
- Former partner at a Boston-based international law firm and co-chair of its patent procurement practice group.
- Graduate studies in chemistry and worked as a synthetic organic chemist for E.I. DuPont de Nemours Co. (now part of Bristol-Myers Squibb).

NEURAEGIS
COMPETITIVE ADVANTAGE

COMPETITIVE ADVANTAGE

NEURAEGIS TAKES A DIFFERENTIATED APPROACH

NeurAegis

NeurAegis's selective calpain-2 inhibitor, NA184, is differentiable from current TBI treatments as well as other products in development:

Current TBI treatment options include psychostimulants, antidepressants, antiparkinsonian agents, and anticonvulsants

PNT001 (Pinteon)	AST-004 (Astrocyte Pharm)	N-Acetyl-cysteine (Neuronasal)	OXE-103 (Oxeia)	VAS203 (Vasopharm)	NeuroSTAT (NeuroVive Pharmaceutical)	CEVA101 (Cellvaton)
Tau antibody	*Adenosine receptor agonist*	*Antioxidant*	*Ghrelin*	*Inhibitor of nitric oxide synthase*	*Proprietary formulation of cyclosporin; targeting mitochondria*	*Autologous bone marrow-derived mononuclear cells (bmmncs)*

FAVORABLE MARKET DYNAMICS

- » TBI drugs in pipeline with novel MoA are still **years from commercialization** - expected in the 2024-2026 timeframe
- » Potential for **relatively short development** and regulatory timelines
- » **Blockbuster potential** for first in class or best in class therapeutics, and room for multiple "**winners**"
- » In 2019, **NeuroSTAT** received **Fast Track designation** from the FDA
- » **VAS203** expected to reach peak revenues of **$2.5B by 2030**

SERIES A FINANCING

NeurAegis

PROPOSED TERMS FOR SERIES A RAISE

HIGHLY PROMISING INVESTMENT FOR NEW ENTRANTS

NeurAegis

» **$10M Series A Preferred financing**

- Funds to key milestones/value inflection points (Ph 1in concussion)
- 2 years operating runway

» **Fully diluted pre-money valuation= $15M**

» **Liquidation preference**

- First pay one times the Original Purchase Price plus declared and unpaid dividends on each share of Series A Preferred; (or, if greater, the amount that the Seed Preferred would receive on an as-converted basis). The balance of any proceeds shall be distributed pro rata to holders of Common Stock.

» Weighted average **anti-dilution protection**

» **One Board or Observer seat** (if desired) for Series A Preferred investor class

COMPARABLES

ACTIVE ACQUISITION MARKET IN THE NEURAL FIELD

NeurAegis

cerevel

» A clinical-stage biopharmaceutical company that combines a deep understanding of the biology and neurocircuitry of the brain?

» Focused on the discovery, development and commercialization of innovative new treatments for the neurodegenerative community PD, AD, Movement disorder.

» *Acquired By: Abbvie Aug 2024, $8.7 Billion*

NeuroDerm

A Mitsubishi Tanabe Pharma Group Company

» Is working on innovative solutions for Parkinson's disease treatment

» NeuroDerm's flagship product ND0612 is currently in late stage development for Parkinson's disease patients experiencing motor fluctuations.

» *Acquired By: Mitsubish Tanable Pharma, 2017 $1.1 Billion*

remedy PHARMACEUTICALS

» Intravenous formulation for acute central nervous system conditions such as stroke (ischemic and hemorrhagic), traumatic brain injury.

» *Acquired By: Biogen May 15, 2017 $120M*

COMPARABLES

AVERAGE MARKET CAP AT THE TIME OF IPO IS $570 MILLION

NeurAegis

Compagnies involved in developing treatments for neurological disorders and have gone public in the last few years. Their average market capitalization at the time of IPO is $570 million.

Company Name	***Stock Ticker***	***IPO Date***	***Market Capital at IPO*** ***(USD)***
Amylyx Pharmaceuticals	NASDAQ:AMLX	Jan 7, 2022	1,069,196,253
Longboard Pharmaceuticals	NASDAQ:LBPH	Mar 12, 2021	270,671,840
Foghorn Therapeutics	NASDAQ:FHTX	Oct 23, 2020	571,116,832
Ovid Therapeutics	NASDAQ:OVID	May 5, 2017	369,029,055

SUMMARY OF INVESTMENT OPPORTUNITY
VALIDATED KPIs FOR INVESTORS' ANALYSIS

NeurAegis

NeurAegis's therapeutics have the potential to transform treatment of TBI and other neurological disorders and to achieve blockbuster status ($1+ billion in annual revenues)

» Novel therapeutics with **blockbuster potential**

- **First-in-class** to prevent neuronal death
- Addressing **multiple neurological disorders**, large markets
- **No current FDA-approved medication**—huge unmet medical need!
- **Plasma biomarker**

» Strong **Intellectual Property**

» Experienced **Leadership Team**

» Reduced **clinical risk**

» Significant Upside **Potential**

- **Key milestones achieved** on grant funding (validative and non dilutive)
- First **institutional round**
- Exit comparables suggest **10x return potential**

NeurAegis

For further details

Please contact

Stella M. Sung, Ph.D.
President & CEO

SSUNG@NEURAEGIS.COM

WWW.NEURAEGIS.COM

APPENDIX

INTELLECTUAL PROPERTY

NeurAegis

Western University OF HEALTH SCIENCES

» Exclusive license with Western University of Health Sciences

» Multiple pending patent applications
  - Composition of matter and therapeutic uses of calpain inhibitors
  - Novel blood biomarker for diagnosis and monitoring of acute neuronal damage
  - Potential process patents for lead compounds

» Received favorable International Search Report and Written Opinion for composition of matter claims

KEY MILESTONES

NeurAegis

Accomplished

2016 - 2017
- Company formation
- Identified calpain-2's role in neurodegeneration

2018 - 2019
- Discovered novel calpain-2 protease inhibitors (NCE's)
- Demonstrated efficacy in several preclinical animal models
- Obtained $3.8M DOD grant

2020
- Selected clinical candidate for concussion (NA184)
- Identified and screened novel compounds for additional indications

2021 - 2023
- Initiate IND-enabling program for concussion/TBI

2023
- Awarded >$1M in additional grant funding, including NIH grant for status epilepticus

Up Next

2024
- Complete pre-IND studies for concussion
- Secure funding for first-in-human clinical studies (concussion) and earlier stage programs

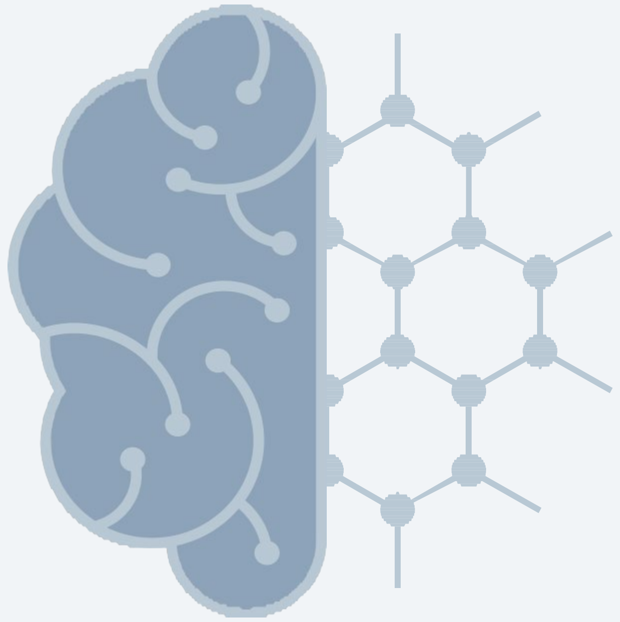
2024 - 2025
- Select clinical candidate for status epilepticus (most severe form of epilepsy)
- Initiate Phase 1 studies for concussion
- Initiate IND-enabling program for status epilepticus

2026
- End of Phase 1

STIMULATION OF NEUROGENESIS AS A POTENTIAL TREATMENT FOR NEURODEGENERATIVE DISEASES

NeurAegis

01 ***Neurogenesis is altered in neurodegenerative diseases (AD, PD, etc.)***

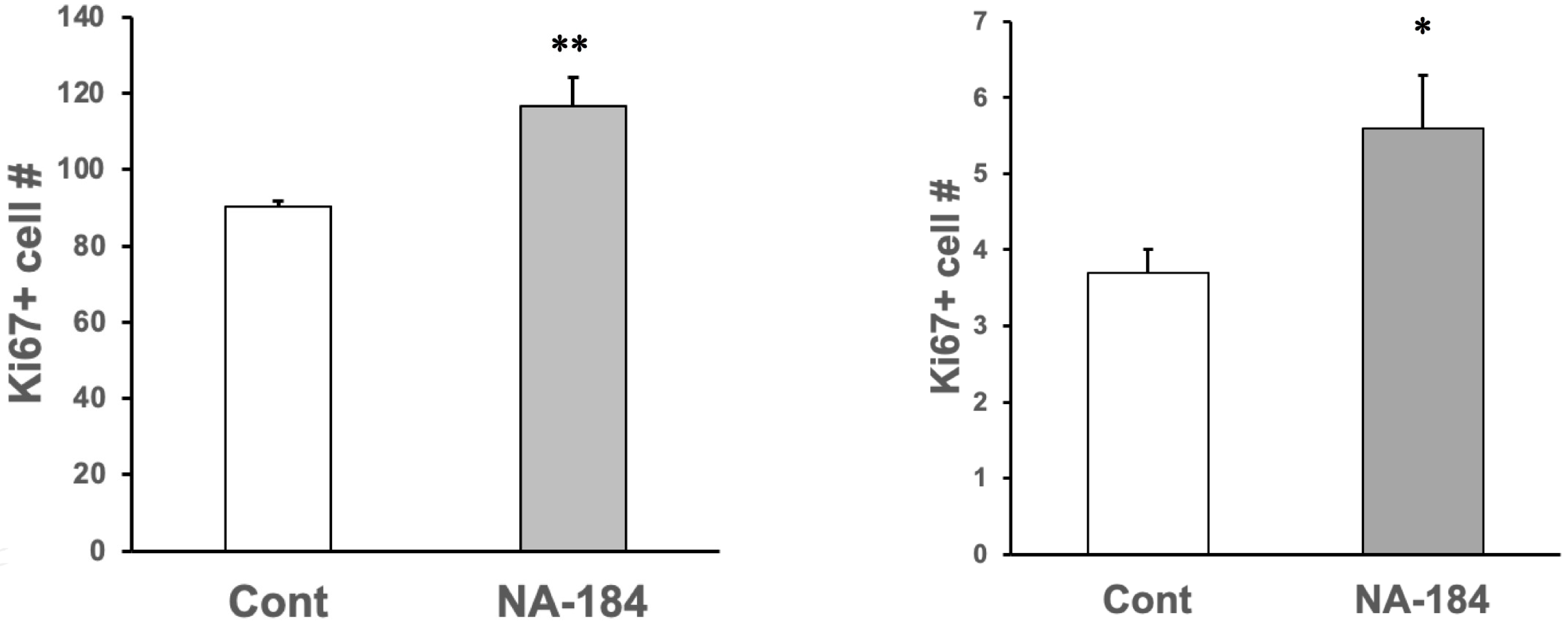


02 ***Enchancement of neurogensis is beneficial in aging and neurodegeration***

03 ***Additional benefit of a selective calpain-2 inhibitor: stimulation of neurogenesis in adult mouse brain***

INHIBITOR: NA-184

STIMULATES NEUROGENESIS

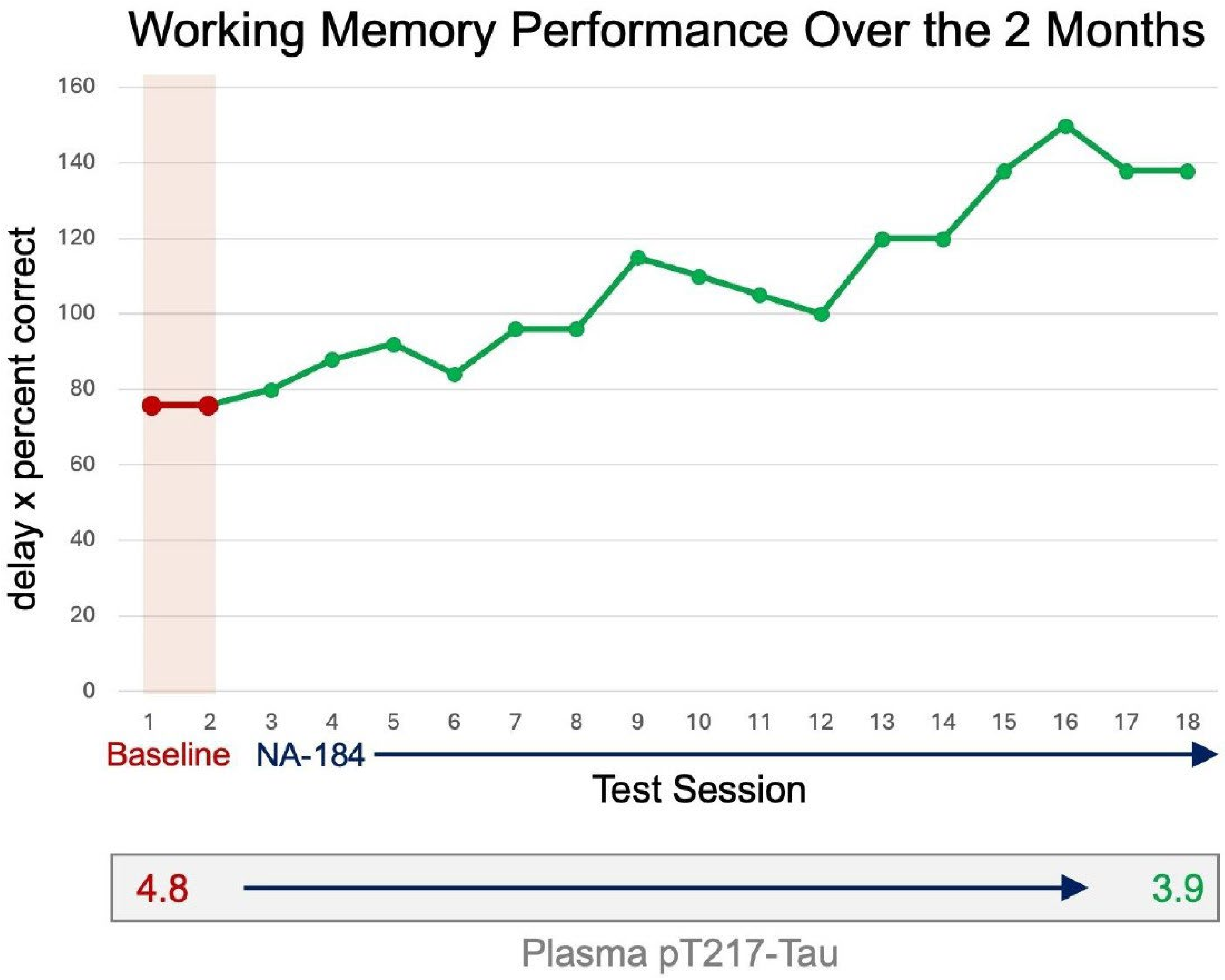


SVZ ** $p < 0.005$

DG * $p < 0.05$

DAILY INJECTIONS OF NA-184 FOR 2 MONTHS IN AGED MONKEYS INCREASE LEARNING AND MEMORY AND DECREASE PLASMA P-TAU

NeurAegis



Ref: Appendix p 35, 36

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